

Clinical Reference Guide: Vulvar Cancer

Extracted Focus & Question-Answer Summary from Cancer.gov PDQ

DOCUMENT OVERVIEW

Focus Condition:	Vulvar Cancer
Semantic Group:	Disorders (T191)
Source Data:	National Cancer Institute (NCI) / CancerGov

Question 1: What is (are) Vulvar Cancer?

Vulvar cancer is a rare disease in which malignant (cancer) cells form in the tissues of the external genitalia, most commonly affecting the outer vaginal lips. Less frequently, it affects the inner vaginal lips, clitoris, or vaginal glands. It usually develops slowly over a number of years, often starting as a precancerous skin condition known as **vulvar intraepithelial neoplasia (VIN)**. Because VIN can progress to invasive vulvar cancer, early evaluation and treatment are critical.

Risk Factors: Having a risk factor does not mean an individual will get cancer, but factors that increase risk include:

- Having vulvar intraepithelial neoplasia (VIN).
- Having human papillomavirus (HPV) infection.
- Having a history of genital warts or abnormal Pap tests (Pap smears).
- Having multiple sexual partners or initiating sexual intercourse at a young age.

When the disease returns after completing initial treatment, it is diagnosed as **recurrent vulvar cancer**, which may manifest in the vulva or other parts of the body.

Question 2: What are the symptoms of Vulvar Cancer?

Vulvar cancer often does not cause early signs or symptoms. When symptoms do manifest, they may be caused by the malignancy or by benign medical conditions. Patients should consult a doctor if they experience:

- A palpable lump, nodule, or growth on the vulva.
- Visible alterations in vulvar skin, such as color changes or lesions resembling a wart or ulcer.
- Persistent itching (pruritus) in the vulvar area that does not resolve.
- Abnormal bleeding unrelated to menstruation.
- Local tenderness or pain in the vulvar region.

Question 3: How to diagnose Vulvar Cancer?

Specialized tests and clinical examinations are required to detect and diagnose vulvar cancer accurately:

- **Physical Exam and Medical History:** A comprehensive examination of general health, personal health habits, and past medical treatments, alongside a detailed physical inspection of the vulva for unusual lumps or skin changes.
- **Biopsy:** The definitive diagnostic procedure involving the surgical removal of specific cell or tissue samples from abnormal areas of the vulva. These samples are examined under a microscope by a pathologist to identify malignant cells.

Question 4: What is the outlook for Vulvar Cancer?

The prognosis (chance of recovery) and the selection of appropriate treatment pathways depend significantly on several clinical factors:

- **Cancer Stage:** How deeply the tumor has invaded tissue and whether it has spread to lymph nodes or distant organs.
- **Patient Profile:** The patient's age, overall physical condition, and medical history.
- **Disease Status:** Whether the malignancy is a newly diagnosed primary tumor or a recurrence of a previously treated cancer.

Question 5: What are the stages of Vulvar Cancer?

Once diagnosed, staging procedures (such as pelvic exams, colposcopy, cystoscopy, proctoscopy, CT/MRI/PET scans, and sentinel lymph node biopsy) determine the extent of disease spread. The recognized clinical stages include:

- **Stage 0 (VIN / Carcinoma in situ):** Abnormal cells are localized entirely to the surface layer of the vulvar skin and have not invaded deeper tissue.
- **Stage I:** Cancer has formed and is confined solely to the vulva or perineum.
 - *Stage IA:* Tumor is ≤ 2 cm in size and has invaded tissue to a depth of ≤ 1 mm. No lymph node involvement.
 - *Stage IB:* Tumor is > 2 cm in size OR has invaded tissue > 1 mm deep. No lymph node involvement.
- **Stage II:** Tumor of any size that has grown directly into adjacent structures (lower urethra, lower vagina, or anus), without spreading to lymph nodes.
- **Stage III:** Tumor of any size with or without invasion into adjacent lower structures, but accompanied by metastasis to one or more nearby inguinal/femoral lymph nodes (subdivided into IIIA, IIIB, and IIIC based on node count and size).

- **Stage IV:** Advanced metastatic disease.
 - *Stage IVA:* Cancer has invaded the upper urethra, upper vagina, bladder mucosa, or rectal mucosa; is fixed to pelvic bone; or involves ulcerated/fixed lymph nodes.
 - *Stage IVB:* Distant metastasis to pelvic lymph nodes or distant organs (e.g., lungs or bone).

Question 6: What research (or clinical trials) is being done for Vulvar Cancer?

Extensive research is conducted through clinical trials to evaluate whether new therapies, drug combinations, or surgical techniques are safer and more effective than current standard treatments.

Patients may enroll in clinical trials before starting initial therapy, during standard treatment, or after standard options have proven ineffective. Participation directly contributes to advancing oncology protocols and improving future cancer care. Information regarding active NCI-supported clinical trials is publicly accessible through official registries.

Question 7: What are the treatments for Vulvar Cancer?

Standard management of vulvar cancer utilizes four primary therapeutic modalities, often combined based on stage and anatomical location:

- **Surgery:** The most common primary treatment, aimed at complete tumor resection while preserving maximum sexual function. Options range from laser surgery, wide local excision, and radical excision to various extents of vulvectomy (skinny, modified radical, or radical) and pelvic exenteration for advanced disease. Sentinel lymph node biopsy is often used to assess lymphatic spread with minimal morbidity.
- **Radiation Therapy:** Uses high-energy x-rays or radioactive implants (external or internal radiation) to eradicate cancer cells. Often administered as adjuvant therapy post-surgery to reduce recurrence risk, or as palliative care for symptom relief.
- **Chemotherapy:** Systemic or regional drug therapies designed to halt cancer cell division. May be combined with radiation (chemoradiation) before surgery or for advanced/recurrent cases. Topical formulations (e.g., creams) are sometimes used for pre-invasive lesions.
- **Biologic Therapy (Immunotherapy):** Utilizes substances to stimulate or restore the patient's immune defenses against cancer cells. Topical imiquimod cream is a notable biologic therapy utilized for certain surface lesions and VIN.
- **Follow-Up Care:** Ongoing, structured medical surveillance is mandatory after treatment completion to monitor treatment efficacy, manage side effects, and detect potential recurrences early.